

**HOW TO USE THIS SET**

20 questions, mixed difficulty, 4 case-based. Attempt every question before checking the answer key on the last page.

● 4 Easy

● 10 Medium

● 6 Hard

1.**EASY**

Primary biliary cholangitis is most strongly associated with which antibody?

- A. Antimitochondrial antibody
- B. Anti-smooth muscle antibody
- C. Anti-Jo-1
- D. ANCA
- E. Anti-dsDNA

2.**EASY**

Which disease is primary sclerosing cholangitis most commonly associated with?

- A. Hereditary hemochromatosis
- B. Systemic lupus erythematosus
- C. Ulcerative colitis
- D. Celiac disease
- E. Crohn disease of the stomach

3.**EASY**

What are the two main types of gallstones?

- A. Cholesterol stones and pigment stones
- B. Calcium phosphate and uric acid stones
- C. Mixed and triple phosphate stones
- D. Struvite and cystine stones
- E. Oxalate and urate stones

4.**EASY**

What is the commonest form of acute cholecystitis?

- A. Xanthogranulomatous cholecystitis
- B. Acute calculous cholecystitis
- C. Emphysematous cholecystitis
- D. Acute acalculous cholecystitis
- E. Gangrenous cholecystitis without stones

5.**MEDIUM**

Which histologic finding is classic for primary sclerosing cholangitis?

- A. "Onion-skin" periductal fibrosis
- B. Pseudoacinar hepatocyte architecture
- C. Copper deposition in hepatocytes
- D. Florid duct lesions with granulomas only
- E. Mallory-Denk bodies

6.**MEDIUM**

What imaging finding is characteristic of primary sclerosing cholangitis?

- A. A single smooth extrahepatic stricture only
- B. Normal-appearing bile ducts
- C. Beading of the intra- and extrahepatic biliary tree
- D. Diffuse hepatic steatosis
- E. A solitary dilated gallbladder with a normal biliary tree

7.

MEDIUM

Which process drives cholesterol gallstone formation when bile cholesterol exceeds its solubilizing capacity?

- A. Increased lecithin solubilizing all cholesterol
- B. Rapid gallbladder emptying preventing nucleation
- C. Supersaturation causing cholesterol to crystallize out of solution
- D. Excess bile acid synthesis diluting cholesterol
- E. Decreased mucus secretion

8.

MEDIUM

Which factor enhances cholesterol gallstone formation by promoting crystal nucleation?

- A. Increased gallbladder contraction
- B. Low biliary cholesterol
- C. Rapid bile flow
- D. Gallbladder hypomotility/stasis
- E. Decreased mucus secretion

9.

MEDIUM

Brown pigment gallstones are characteristically found in association with:

- A. Oral contraceptive use
- B. Rapid weight loss alone
- C. Infected intrahepatic or extrahepatic ducts
- D. Sterile gallbladder bile with intravascular hemolysis
- E. Hereditary hemochromatosis

10.

MEDIUM

CASE

A 55-year-old woman develops obstruction of the cystic duct by a gallstone, with right upper quadrant pain, fever, and leukocytosis.

This presentation is best classified as:

- A. Cholangiocarcinoma
- B. Acute acalculous cholecystitis
- C. Primary sclerosing cholangitis
- D. Acute calculous cholecystitis
- E. Chronic cholecystitis alone

11.

MEDIUM

Which organisms are most commonly cultured from the bile in chronic cholecystitis?

- A. Salmonella typhi
- B. E. coli and enterococci
- C. Candida species
- D. Pseudomonas and Klebsiella only
- E. Staphylococcus aureus and Streptococcus pyogenes

12.

MEDIUM

What proportion of gallbladder carcinoma cases are associated with gallstones?

- A. About 10%
- B. About 70%
- C. About 50%
- D. About 25%
- E. About 95%

13.

MEDIUM

Microscopically, most gallbladder carcinomas are of which histologic type?

- A. Adenocarcinoma
- B. Squamous cell carcinoma
- C. Small cell carcinoma
- D. Lymphoma
- E. Sarcoma

14.

MEDIUM

Which risk factor is shared by both gallbladder carcinoma and cholangiocarcinoma?

- A. Oral contraceptive use
- B. Primary sclerosing cholangitis
- C. Hereditary hemochromatosis
- D. Wilson disease
- E. Alpha-1-antitrypsin deficiency

15.

HARD CASE

A 48-year-old man with a history of chronic liver fluke infestation develops a mass at the confluence of the left and right hepatic ducts.

Based on its location, this cholangiocarcinoma is best classified as:

- A. Ampullary carcinoma
- B. Gallbladder carcinoma
- C. Distal CCA
- D. Intrahepatic CCA
- E. Perihilar CCA

16.

HARD

Which combination of features best distinguishes PBC from PSC?

- A. PBC spares the extrahepatic biliary tree and is AMA-positive; PSC involves intra- and extrahepatic ducts and is typically AMA-negative
- B. PBC predominantly affects young men; PSC predominantly affects middle-aged women
- C. PBC shows "onion-skin" fibrosis; PSC shows florid duct lesions
- D. PBC and PSC are histologically indistinguishable
- E. PBC is strongly linked to ulcerative colitis; PSC is strongly linked to Sjögren syndrome

17.

HARD CASE

A patient with long-standing gallstones and recurrent bouts of gallbladder inflammation is found at cholecystectomy to have a poorly defined area of diffuse gallbladder wall thickening and induration.

This gross growth pattern of gallbladder carcinoma is best described as:

- A. Papillary only
- B. Cystic
- C. Exophytic
- D. Infiltrative
- E. Polypoid

18.

HARD

A mixed cholesterol gallstone is radiopaque mainly because of which component?

- A. Uric acid crystals
- B. Calcium salts admixed with cholesterol
- C. Pure crystalline cholesterol monohydrate
- D. Cystine content
- E. Bilirubin calcium salts alone

19.

HARD CASE

A patient with longstanding gallstones develops progressive biliary obstruction. Imaging shows a stone that has migrated from the gallbladder into the common bile duct, with dilation of the intrahepatic ducts and fever.

Which complication best explains this presentation?

- A. Acute acalculous cholecystitis
- B. Primary biliary cholangitis
- C. Simple biliary colic
- D. Gallbladder mucocele
- E. Choledocholithiasis with ascending cholangitis

20.

Intrahepatic cholangiocarcinoma may present with which growth patterns?

- A.** Unifocal, multifocal, or diffusely infiltrative only
- B.** Infiltrative or exophytic only
- C.** Pure nodular pattern only
- D.** Mass-forming, periductal-infiltrating, or intraductal-growing
- E.** Cystic pattern only

ANSWER KEY & RATIONALE

Don't peek until you've committed to an answer on every question.

Q	Ans	Why
1	A	Antimitochondrial antibodies (AMA) are present in 90-95% of PBC patients.
2	C	Inflammatory bowel disease, most commonly ulcerative colitis, coexists in approximately 70% of individuals with PSC.
3	A	Gallstones are of two main types: cholesterol stones (crystalline cholesterol monohydrate) and pigment stones (bilirubin calcium salts).
4	B	Acute calculous cholecystitis is the commonest form, seen in about 90% of cases, caused by a gallstone obstructing the gallbladder neck or cystic duct.
5	A	The classic histologic finding in PSC is "onion skin" fibrosis concentrically around affected bile ducts.
6	C	Irregular biliary strictures and dilations cause the characteristic "beading" of the intrahepatic and extrahepatic biliary tree seen on MRI in PSC.
7	C	When cholesterol concentration exceeds the solubilizing capacity of bile (supersaturation), it can no longer stay dispersed and crystallizes out of solution.
8	D	Hypomotility/stasis of the gallbladder promotes crystal nucleation, enhancing cholesterol gallstone formation.
9	C	Brown pigment stones are found in infected intrahepatic or extrahepatic ducts, are few in number, and have a soft, greasy, soap-like consistency.
10	D	Acute calculous cholecystitis, the commonest form, is caused by a gallstone obstructing the gallbladder neck or cystic duct, presenting with RUQ pain, fever, and leukocytosis.
11	B	Microorganisms, usually E. coli and enterococci, can be cultured from the bile in about one-third of chronic cholecystitis cases.
12	E	Gallstones are present in 95% of cases of gallbladder carcinoma, possibly as a result of chronic inflammation.
13	A	Most gallbladder carcinomas are adenocarcinomas; about 5% are squamous cell carcinomas.
14	B	Primary sclerosing cholangitis is a risk factor for both gallbladder carcinoma and cholangiocarcinoma, via chronic biliary inflammation.
15	E	Perihilar CCA arises from the left, right, or common hepatic ducts and is the most common location (50-60% of cases); liver flukes are a recognized risk factor for CCA.
16	A	PBC selectively destroys small/medium intrahepatic ducts (sparing the extrahepatic tree) and is AMA-positive in 90-95%; PSC involves both intra- and extrahepatic ducts, shows "onion-skin" fibrosis, and is usually AMA-negative.
17	D	Gallbladder carcinoma may grow as a poorly defined area of diffuse wall thickening and induration (infiltrative pattern) or as an irregular exophytic mass into the lumen.
18	B	Mixed cholesterol stones contain at least 60% cholesterol with the rest usually calcium salts, which makes them radiopaque, unlike pure cholesterol stones which are often radiolucent.
19	E	Stones that migrate out of the gallbladder into the common bile duct (choledocholithiasis) can obstruct bile flow and lead to ascending cholangitis, with fever and intrahepatic ductal dilation.
20	D	Intrahepatic CCA may present as mass-forming, periductal-infiltrating, or intraductal-growing, based on its site among the bile ductules and segmental ducts.

Pathology GIT Block · MEDucation by Mattin Majid · MCQs written to test SGL6 content — not sourced from a past paper.

Recalled and source questions are treated as evidence of what gets asked, not as a source of correct answers — verify against your lecture notes.